جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : سعاد يوسف حسن عبد الظاهر
المحافظة : الجيزه **الحي / المدينة :** البدرشين **العنوان :** =
رقم البطاقة : 28601112101429 **رقم التليفون :** 01125231544 **تاريخ تسجيل الإستمارة :** 24-10-2026
العمر(تاريخ الميلاد) : 11-01-1986 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مسح ذري علي الجسم (PET SCAN / CT) [8530] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: سعاد يوسف حسن عبد الظاهر
National ID: 28601112101429

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Whole-body PET/CT scan

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

07/22/2026

MDT FORM

Name:	Soaad Yousef Hassan Abd Elzaher	MRN:	131
Age:	41 Y	ID:	28601112101429
Menopaus Status:		Phone:	01125231544
NRS SCORE:	PS 1	Marital Status:	
BMI:		Family History:	
CO Morbidity:	FREE	Reason Of MDT:	METASTAIC WITH NEWELY DEVELOPED

Clinical Data:

BX:

ER 7/8 **PR** 7/8 **HER** +VE **K16** 40

Sonomammography:

Mets Workup:

MRI BRAIN DONE >> TWO POSTERIOR FOSSA SOL LARGERS 2.6*2.4 CM IN THE VERMIS , SMALLER IN

History:

A CASE OF RT BREAST CANCER SINCE 11/2021
RECEIVED NACTH TCHP (TC 6 CYCLES AND ONLY 1 CYCLE HP) THEN RT I1RM IN 6/2022 PATHOLOGY PT0N2M0
COMPLETED 1 YEAR OF HP ENDED 6/2023 , FORTH THEN TAM ZOLADEX

5/2024 PT DEVELOPED BONE AND LUNG METS
PT RECEIVED RTH TO DORSAL 12 AND CTH TAXOTERE HERCEPTIN TILL 12/2024 FOLLOWED BY PU TILL 4/2025
CT CAP SHOWED HFL AND LT BREAST MASS RETROAREOLAR
PT STARTED TYKERB XELODA 5/2025 TILL 3/2026
CT CAP REVEALED STATIONARY COURSE AND CONTINUED TTT
BUT IN APRIL PT COMPLAINED OF SKIN NODULES AT THE SAME SIDE OF MRM
(LOCAL RECURRENCE) THAT APPEARED A MONTH AGO >> PT SHIFTED TO NAVILBINE SINGLE AGENT (6\6\2026) WITH TRIAL OF KADCYLA FROM AGO

Recommendations:

ADD PLATINUM BASED CTH TO NAVILBINE
WCI >> REQUESTED

MDT Outcome & Plan

For:
1.Biopsy from skin nodules for IHC
2.Local field Rth to 2 brain SOLs
3.Add Carboplatin to Navelbene with trial of anti-

R L



TNM

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: سعاد يوسف حسن عبدالظاهر **MR. :** 131	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : | **تاريخ وساعة الخروج :**

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient has chronic kidney disease and anemia and is receiving treatment for de novo breast cancer with a combination of Ibrance and Faslodex. Zometa was discontinued because of the chronic kidney disease.

The patient's oncologic history includes a wide local excision in November 2021, followed by neoadjuvant chemotherapy with TCHP in 2022, then adjuvant TC × 6 plus one cycle of high-dose paclitaxel and radiation therapy to the chest wall (RT-MRM) completed in June 2022. Pathology was T0N2. In August 2023 the patient was started on hormonal therapy with fulvestrant (Faslodex). Metastatic disease to bone and lung was identified, and the patient received taxotere plus trastuzumab in December 2024, with follow-up until April 2025. A CT of the chest, abdomen, and pelvis (CT CAP) showed progressive disease (PD) in the lungs, hepatic focal lesions (HFL), and a local recurrence. Multidisciplinary team (MDT) discussion recommended treatment with Tykerb (lapatinib) and Xeloda (capecitabine) (Tykerb 1 mg × 5 days, Xeloda 3 × 2 weeks). A repeat CT CAP in November 2025 demonstrated stable disease (SD). Laboratory studies were accepted.

A subsequent CT CAP was pending; the treatment plan included continuation of Xeloda 3 × 2 weeks and addition of Tykerb 5 × 1 day for a total of 30 days (C11 Xeloda 3 × 2 weeks + C7 Tykerb 5 × 1 day).

Metastatic breast cancer involving bone and liver has been treated with Tykerb and Xeloda since April 2025. The most recent CT CAP on 14 March 2026 showed a stationary disease course, but the patient developed new skin nodules on the radiation-treated side of the chest wall one month earlier, prompting a new MDT review.

In May 2026 the MDT discussed initiating single-agent vinorelbine (Navilbine) and a trial of Kadcyla (ado-trastuzumab emtansine) sourced from an NGO. Vinorelbine 90 mg was administered on day 1 and day 8, with the next clinic visit scheduled for 29 June 2026.

Laboratory results were acceptable for vinorelbine 90 mg, and the Kadcyla trial from the NGO remained pending. MRI of the brain was scheduled for June 2026. Two new lesions were identified for MDT discussion and consideration of adding platinum-based chemotherapy together with vinorelbine.

The MDT review on 22 July 2026 noted that laboratory results for vinorelbine 90 mg were still pending. The patient reported clinical progression of the breast lesion on CT CAP.

On 12 August 2026 the MDT plan included: (1) biopsy of the skin nodules for immunohistochemistry, (2) local field radiation to the right thoracic region for two brain lesions, and (3) addition of carboplatin to vinorelbine with a trial of anti-HER2 therapy through the NGO. The first cycle of vinorelbine plus carboplatin (AUC 5) was scheduled for 30 July 2026.

تحريرا في ,

التاريخ : 21-09-2026 **الوقت :**

الطبيب المعالج : نهي سعيد علي محمد

يعتمد ,

On 30 August 2026 the patient received a loading dose of Herceptin (8 mg/kg) after an echocardiogram, followed by the second dose of vinorelbine and carboplatin (AUC 5) after laboratory confirmation on 2 September 2026. The patient's weight is 54 kg and height 160 cm.

On 3 September 2026 the patient presented with pre-menopausal metastatic breast cancer involving bone, lung, liver, and locally recurrent disease with chest-wall skin nodules and left-breast induration. A new skin biopsy for IHC was ordered, and the patient obtained approval for Kadcyla. The patient had received recent chemotherapy three days ago and was scheduled for a two-week rest period before starting Kadcyla, with laboratory evaluation showing an ejection fraction of 58 % on 31 August 2026.

New laboratory studies were requested on 12 September 2026 in preparation for cycle 1 of Kadcyla sourced from the NGO. The patient has completed 1 cycle of Kadcyla to date.

معمل حلوان للتحاليل الباثولوجية و الانسجة

ا.د. سامية محمد جبل
دكتوراه واستشاري الباثولوجيا
أستاذ الباثولوجيا كلية الطب جامعة القاهرة

ا.د. سحر محمد طلعت
دكتوراه واستشاري الباثولوجيا
أستاذ الباثولوجيا كلية الطب جامعة القاهرة

PATHOLOGY REPORT

Name: سعاد يوسف
Age: Y
Specimen: Breast mass.

No:782/2021
Referred by: Prof.Dr. Saeid
Date:15/11/2021

GROSS:

Fibrofatty tissue piece measuring 6x4x3 cm., sectioning of which revealed a fairly defined mass with firm grayish lobulated cut section showing gritty rough yellowish foci and streaks.

MICROSCOPIC:

Examination of sections prepared from the specimen received revealed breast tissue infiltrated by malignant tumor tissue formed of sheets of malignant ductal cells, with moderate anaplasia and areas of necrosis. The tumor groups are surrounded by desmoplasia. Foci of high grade duct carcinoma in situ are seen in abcut 5% of the tumor surface area. No evidence of vascular invasion. The resection margins are free of the tumor tissue.

DIAGNOSIS:

BREAST MASS, EXCISION:

- ***INFILTRATING DUCT CARCINOMA, GRADE II.***
- ***FREE RESECTION MARGINS.***

Four slides enclosed.

Prof. Dr. Samia M. Gabal
Samia Gabal

٢٧ ب شارع اسماعيل كامل- خلف عمارات المروة- حلوان ت:٢٨١٦٠٥٢٥ -٠١١٤٩٠١٩٥١٠

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY
INTERNATIONAL
EGYPTE

Patient Name: Souad Youssef Hassan Abdel Zaher	Date : 08/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 39 Year(s)
Medical ID: 5077/2021	Pat. ID: 10460

Post-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED:

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History of right MRM, Compared to previous study dated 27/2/2025 current study revealed:

Chest:

- Right MRM with operative bed showing no sizable masses or collection.
- Clear left breast.
- No suspicious axillary lymph nodes.
- Progressive course as regards the right sided pleural effusion become moderate compared to mild in previous study.
- Still seen Fibrotic bands are seen involving apical segment of right upper lung lobe
- Regressive course as regards the consolidative patch at posterior segment of right lower lung lobe with air bronchogram seen within
- Still seen rather unchanged multiple bilateral pulmonary nodules ranging 0.5-`1 cm in maximum dimensions
- No sizeable pathologically enlarged hilar or mediastinal lymph nodes.
- Normal enhanced CT appearance of the heart and major thoracic vessels

Abdomen and pelvis:

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

Patient Name: Souad Youssef Hassan Abdel Zaher	Date : 08/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 39 Year(s)
Medical ID: 5077/2021	Pat. ID: 10460

- The liver is mildly enlarged in size with still noted small hypodense hepatic focal lesion measures about 1.3x1.2 cm at segment VII.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- No gall bladder hyperdense stones
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No sizeable pathologically enlarged abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

The scanned skeleton:

- D12 vertebral body decrease vertical height with depressed upper end plates and showing heterogeneous mixed sclerotic lytic density
- Still depicted linear sclerotic focus at right iliac bone.
- Left inferior pubic ramus mixed lytic and sclerotic lesions.

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

Patient Name: Souad Youssef Hassan Abdel Zaher	Date : 08/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 39 Year(s)
Medical ID: 5077/2021	Pat. ID: 10460

OPINION:

- **Stationary course of bilateral variable sized Multiple metastatic pulmonary nodules.**
- **Regressive course of right lower lung lobe consolidative patch.**
- **Progressive course as regards the right sided pleural effusion become moderate compared to mild in previous study.**
- **Stationary course of segment VII hepatic focal lesion**
- **Stationary course of D12 mixed lytic and sclerotic lesion and sclerotic right iliac bone.**
- **Newly developed left inferior pubic ramus mixed lytic and sclerotic lesions.**

Dr. Mai Shaaban MD

Dr. Mai Shaaban

Dr. Mai Shaaban

Signature Dr.Mohamed Hassan
11/11/2025 4:23 AM

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient Name: Souad Youssef Hassan Abdel Zaher	Date : 27/02/2025
Exam : C.T CHEST WITH CONTRAST	Age : 39 Year(s)
Medical ID: 5077/2021	Pat. ID: 10460

- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

The scanned skeleton:

- D12 vertebral body decrease vertical height with depressed upper end plates and showing heterogeneous mixed sclerotic lytic density
- Still depicted linear sclerotic focus at right iliac bone.

OPINION:

Status post right MRM showing:

- ***Stationary course*** of bilateral variable sized Multiple pulmonary nodules, Likely metastatic .
- ***Stationary course*** of right lower lung lobe consolidative patch.
- ***Newly developed*** mild right sided pleural effusion.
- ***Newly developed*** hepatic focal lesion
- Stationary course of D12 mixed lytic and sclerotic lesion and sclerotic right iliac bone

Dr. Marwa Kamal Saber

Dr. Walid Mohamed, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]

Cairo university –faculty of medicine
Cairo university hospital
Radiology department

جامعة القاهرة -كلية الطب
مستشفيات جامعة القاهرة
قسم الاشعة التشخيصية

Patient name: SOAD YOUSSIF HASSAN	Age: 36 Years
Modality Name: CT	Study Date: 7/13/2024 11:43:31 AM
Patient id: 510872	

Multislice non contrast CT scan of the chest with multiplanar reformatting revealed:

** **Reported case of cancer breast underwent surgery.**

- Evidence of right mastectomy.
- Multiple bilateral pulmonary variable sized nodules are seen scattered within both lung fields showing related atelectatic bands the largest seen at the posterior segment of the right upper lobe measures about 1.2x0.9 cm in diameter.
- Bilateral upper lobar apical subpleural reticulations likely post old granulomatous infections sequel.
- Few subcentimetric mediastinal lymph nodes.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.
- Heterogenous texture of D12 vertebral body showing lytic areas with structural collapse is noted; for Bone scan correlation.
- CT cuts of the upper abdomen show right hepatic lobe segment VII/VI hypodense focal lesion measures about 1.4x1 cm in diameter for dedicated triphasic study.

OPINION:

- **Multiple bilateral pulmonary nodules as described likely metastatic.**
  - **Heterogenous texture of D12 vertebral body showing lytic areas with structural collapse is noted; for Bone scan correlation.**
-

Dr/Naglaa Mahmoud Ali Dr.Nourhan Mohamed,MD

CT Scan
[[DOCTYPE:CT_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Date : 13/3/2024	Patient Name: Souad Youssef Hassan Abdel Zaher
Age : 38 Year(s)	
	Medical ID:

Plain X-ray Chest (PA view) revealed:

- *Clear both lung fields.*
- *Clear both costo-phrenic angles.*
- *Normal cardiac size & configuration.*
- *No bony chest lesion.*

Best regard

Dr/ Dina Gaber ,FEBR

Not Approved

Ultrasound
[[DOCTYPE:ULTRASOUND]]

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وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Souad Youssef Hassan Abdel Zaher	Date : 27/02/2025
Exam : C.T CHEST WITH CONTRAST	Age : 39 Year(s)
Medical ID: 5077/2021	Pat. ID: 10460

- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

The scanned skeleton:

- D12 vertebral body decrease vertical height with depressed upper end plates and showing heterogeneous mixed sclerotic lytic density
- Still depicted linear sclerotic focus at right iliac bone.

OPINION:

Status post right MRM showing:

- ***Stationary course*** of bilateral variable sized Multiple pulmonary nodules, Likely metastatic .
- ***Stationary course*** of right lower lung lobe consolidative patch.
- ***Newly developed*** mild right sided pleural effusion.
- ***Newly developed*** hepatic focal lesion
- Stationary course of D12 mixed lytic and sclerotic lesion and sclerotic right iliac bone

Dr. Marwa Kamal Saber

Dr. Walid Mohamed, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]

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Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Souad Youssef Hassan Abdel Zaher	Date : 11/11/2024
Exam : C.T CHEST	Age : 39 Year(s)
Medical ID: 5077/2021	Pat. ID: 10460

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Compared to previous study noted at 29/12/2022

Chest:

- Right MRM with operative bed showing no sizable masses or collection.
- **Total resolution** of previously seen right upper lobe consolidative patches
- Fibrotic bands are seen involving apical segment of right upper lung lobe
- **Newly developed** consolidative patch at posterior segment of right lower lung lobe with air bronchogram seen within
- **Newly developed** multiple bilateral pulmonary nodules ranging 0.5-0.7 cm in maximum dimensions
- No sizeable pathologically enlarged hilar or mediastinal lymph nodes.
- No pleural or pericardial effusion
- Normal non enhanced CT appearance of the heart and major thoracic vessels

Abdomen and pelvis:

- The liver is mildly enlarged in size with homogenous CT texture. No definite focal lesions along the basis of the non-enhanced study.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- No gall bladder hyperdense stones
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No sizeable pathologically enlarged abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.

CT Scan
[[DOCTYPE:CT_SCAN]]

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Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Souad Youssef Hassan Abdel Zaher	Date : 11/11/2024
Exam : C.T CHEST	Age : 39 Year(s)
Medical ID: 5077/2021	Pat. ID: 10460

- Normal appearance of the pancreas and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

The scanned skeleton:

- D12 vertebral body decrease vertical height with depressed upper end plates and showing heterogeneous mixed sclerotic lytic density
- Still depicted linear sclerotic focus at right iliac bone.

OPINION:

Status post right MRM showing:

- Multiple pulmonary nodules and heterogeneous D12 vertebral body, likely metastatic deposits
- Newly developed right lower lung lobe pneumonia

Dr. Basma Mahrous

Dr. Ibrahim Abo El Magd, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]